

Recollect — SIH 2026 Spoken Script (Variant 2)

Team ByteForce | PS 26003 | ~10 minutes | 3 Speakers

Same structure and content as the first script, but phrased more conversationally, with a few rhetorical hooks and audience-engagement lines. Pick whichever version feels more natural in your own voice — or mix lines from both.

SPEAKER 1 (~2.5–3 min)

Good morning, judges. We're Team ByteForce, and we want to start with a question: what does "smart" elder care actually look like for someone who's losing their memory?

Because right now, it looks like buzzing smartwatches and cluttered phone apps — built for people who are young and tech-comfortable, not for a 78-year-old dealing with confusion and anxiety every day. That's the gap we're addressing with **Recollect**, our answer to Problem Statement 26003: an AI-based cognitive gaming and memory assistance platform for elderly dementia patients in the North East.

[pause]

Let's break the problem down properly, because it isn't just one person's problem — it's three.

For the **senior** — memory loss isn't just forgetting a name. It's confusion, it's anxiety, it's slowly pulling away from the people around you. And the tools that exist today make that worse, not better.

For the **caregiver** — usually a son or daughter, often not even living in the same town — there's no real visibility. Did dad take his medicine? Is he doing okay today? Right now, honestly, they just don't know. That uncertainty is exhausting, and it's a huge driver of caregiver burnout.

And for the **health system** — our ASHA and ANM workers are doing genuinely heroic work, but they're doing it without any simple tool to track homebound seniors between visits. And the "smart" monitoring systems that do exist? They're often sensor-heavy surveillance setups that actually erode the trust these families need to have.

Now add the North East specifically into this picture — multiple regional languages, patchy connectivity in a lot of areas, and families who realistically have access to one shared, budget Android tablet. So whatever we build has to work **offline**, has to speak the **local language**, and has to run on **hardware people already own**. That's not a wish list — that's the actual bar.

So we designed Recollect around one core philosophy: **calm over clever**. I'll pass it to [Speaker 2] to show you what that actually looks like.

SPEAKER 2 (~3.5–4 min)

Thanks. So think of Recollect less as "one app" and more as three connected experiences, all sitting on top of one shared local database — a senior tablet view, a caregiver portal, and a clinical hub for doctors and ASHA workers.

[click — senior view]

Let's start where it matters most — the senior's screen. It's one tap to enter. No PIN, no login, nothing to fumble with. The app greets them by time of day — good morning, good afternoon — spoken aloud in their own regional language, and deliberately slow and calm. We actually tuned the speech rate and pitch ourselves so it never sounds rushed.

And here's a small detail we're proud of — medicine reminders don't just say "take your pill." They show what the pill *actually looks like* — its colour, its shape, any marking on it — because for someone with memory loss, recognizing something is often far easier than reading a label. When a reminder is due, it's full-screen, with exactly three buttons: "Done," "Remind me in 10 minutes," or "I need help." No clutter. No countdown clock creating pressure.

[click — games]

Now — the cognitive games, which is really the heart of this problem statement. We built four dedicated engines, each targeting a specific cognitive domain:

Memory — photo-matching, using family photos, or a Northeast Cultural Heritage pack — Muga silk, Majuli masks, living root bridges — so the experience feels like *their* world, not a generic app built for someone else.

Attention — a calm visual search, spotting flowers in a garden.

Language — everyday object naming and recall.

Problem-solving — putting the steps of a daily routine back in order.

Every game runs full-screen, with zero timers and zero penalty sounds. Why? Because stress actually *hurts* cognitive performance in dementia patients — so the whole interface is built to never rush anyone.

[click — AI layer]

Here's the AI, and this is the part I want you to really register. Every game session silently logs three signals — accuracy, hesitation latency in milliseconds, and completion time. The patient never feels watched, but that data feeds directly into an **explainable rule engine** that continuously tracks four things: missed-reminder streaks, gaps in activity, decline in game scores over time, and increases in response latency.

We deliberately didn't build a black box here. Instead of an opaque model, this produces clear, evidence-based flags — "Watch" or "Alert" — that a doctor can actually look at and explain to a family. In a rural healthcare context, that kind of explainability isn't a nice-to-have. It's what makes the system trustworthy enough to actually use.

[click — caregiver/clinical]

The caregiver side is PIN-gated — live schedules, overdue-medicine alerts, trend charts over time, and a shared family memory journal so multiple caregivers can leave notes for each other.

And on the clinical side, doctors get a weekly composite score plus those same explainable flags, which they can mark reviewed or dismissed with a tap. ASHA workers get a much simpler mobile checklist, built specifically for low-spec phones — because that's the real hardware reality of last-mile health work here.

And underneath all of it: five regional languages — English, Assamese, Bengali, Khasi, Hindi — fully voice-assisted, high-contrast and large-text accessibility modes, and the entire patient experience works **completely offline**, syncing later whenever a connection is available.

That's the how. Now [Speaker 3] will tell you why this is actually deployable, and what it's worth.

SPEAKER 3 (~3-3.5 min)

Thanks. Let's talk about whether this is actually realistic — because a good idea only matters if it survives contact with a real budget and real hardware in the North East.

Technically — Recollect is a zero-dependency progressive web app, just HTML5, CSS3, and modern JavaScript underneath. It runs on the budget Android tablets families already have. No new hardware purchase needed for a pilot.

Economically — there's a low-power mode that turns off animations and background polling to protect battery life on cheap tablets, and since the core features work offline, there's no ongoing cloud bill tied to every patient interaction.

For safety and operations — access is role-gated: caregivers need a PIN, clinicians need a passcode, so a senior can never accidentally change a setting or lose their own data.

Socially — sensitive health data is stored locally on the device first. In rural households especially, that local-first design is a huge factor in whether families actually trust and adopt this.

[pause]

Now, quickly, the numbers — because we wanted this to be more than just a nice idea.

Nationally, around 8.8 million Indians live with dementia — that's a Total Addressable Market of roughly 0.4 billion dollars for digital cognitive care.

Zooming into the North East — about 3.2 million seniors, applying the national dementia prevalence rate, gives us roughly 237,000 patients, an 11-million-dollar Serviceable Available Market just in this region.

And for a realistic Year-1 pilot through the ASHA and PHC network in Assam and Meghalaya, we're targeting a Serviceable Obtainable Market of about 53,000 dollars. That number is small on purpose — this is a rollout-first plan, not a hype-first one.

[pause]

So, what does this actually change, for the people this problem statement is about?

For **seniors** — a calmer daily life, and independence preserved for longer.

For **caregivers** — real oversight instead of guesswork, which directly eases the burnout we talked about earlier.

For **doctors and ASHA workers** — explainable data instead of flying blind between home visits.

And more broadly — fewer unnecessary clinic trips, lower travel-related emissions, and strong alignment with the Ayushman Bharat Digital Mission and India's digital health goals.

[pause]

So to close — Recollect isn't trying to replace the people who care for our elders. It's trying to make that care reach further — even into a village with a patchy signal and one shared tablet.

We're Team ByteForce. Thank you — happy to take your questions.

Notes on this variant vs. the first script

Same content, same timing targets — this version leans more conversational, with rhetorical questions and a couple of "register this" style emphasis lines for the AI section, since that's where judges tend to probe hardest.

Feel free to mix and match sentences from both scripts — whichever phrasing feels most natural coming out of your mouth is the right one to keep.

Same trim advice applies if you're running long: cut from Speaker 1's "North East specifically" paragraph or Speaker 2's closing accessibility line first.